The product at a glance

Executive visual companion | Patient experience, healthcare value and the planned local deployment.



Source: PMD v2.0 pp. 1-6; user-approved positioning | Visual companion v1.0 | 8 October 2026

AWARE CARE

8 value hypotheses. Evidence before claims.

Benefits span patients, clinical teams, payers and technology leaders. Savings are not measured by demo clicks.

B01 | PATIENTS

Better care destination

Specialty and preference matching.
Measure appropriate first destination.

B02 | CLINICIANS

Less avoidable demand

Reviewed guidance for suitable concerns.
Measure avoidable visits AND delays.

B03 | CLINICAL STAFF

Less repeated intake

Confirmed history and useful summaries.
Measure net time including corrections.

B04 | HEALTH SYSTEMS

More useful capacity

Reduce misdirected appointments.
Measure wait times and filled slots.

B05 | PAYERS

Lower avoidable spending

More appropriate episode utilization.
Measure adjusted total allowed cost.

B06 | CAREGIVERS

Less navigation effort

Clear next steps and visit options.
Measure time, cost and comprehension.

B07 | CIO / PRIVACY

More data control

Local inference and lifecycle controls.
Verify egress, access and deletion.

B08 | CTO / OPERATIONS

Observable AI delivery

Versioned models and repeatable builds.
Measure recovery and cost per run.

Economic discipline

Freed capacity is not automatically cash savings. Provider revenue effects and payer savings must be evaluated separately.

Source: PMD v2.0 pp. 2-4, 35 | Benefits are hypotheses, not proven outcomes

AWARE CARE

1 responsive application. 24 registered screens.

The mobile experience is the responsive layout of the same web routes. Native iOS and Android are future scope.

PUBLIC WEBSITE | 5

W01 Home
W02 How it works
W03 For healthcare
W04 Privacy
W05 Help

PATIENT JOURNEY | 12

A01 Start and consent
A02 Health profile
A03 Describe concern
A04 Records
A05 Adaptive questions
A06 Review your case
A07 Your guidance
A08 Sources
A09 Doctor preferences
A10 Doctor matches
A11 Doctor details
A12 Appointment summary

CONTINUITY + PRESENTER | 7

A13 Consultation history
A14 Data settings
A15 Urgent action
A16 Service limits

O01 Runtime overview
O02 Evidence and controls
O03 Demo scenarios

Every screen includes its states

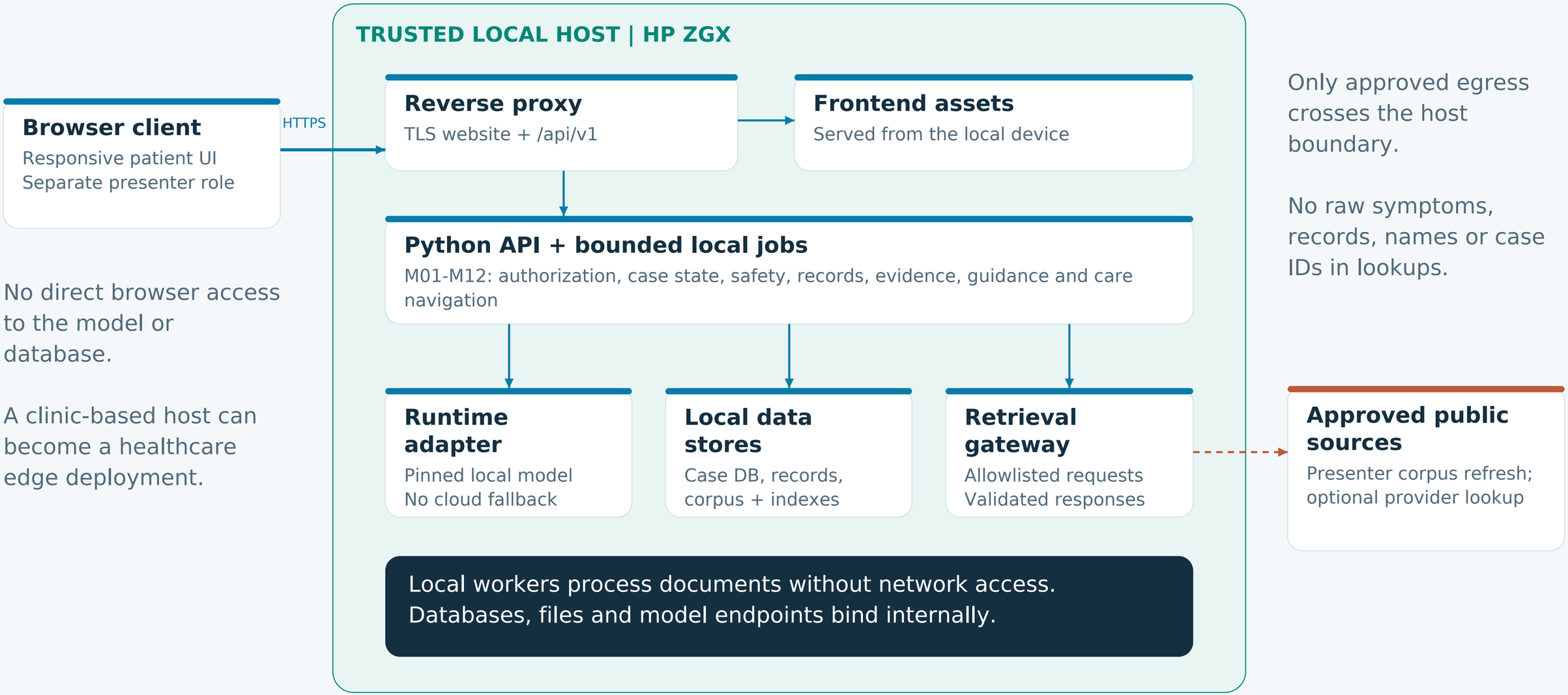
Loading, empty, validation, offline, failure and stale results belong to their owning screen. Direct links enforce the same guards.

Source: PMD v2.0 pp. 6-16 | Screen identities preserved

AWARE CARE

Target deployment: 1 ZGX, explicit boundaries

The reverse proxy exposes the website and API. Internal inference, stores and workers are not exposed to the client.



No direct browser access to the model or database.

A clinic-based host can become a healthcare edge deployment.

Only approved egress crosses the host boundary.

No raw symptoms, records, names or case IDs in lookups.

Source: PMD v2.0 pp. 29, 32, 34 | Planned deployment; no selected database or LLM implied

AWARE CARE

12 logical modules. Clear behavioral ownership.

These are Python packages with explicit service interfaces, not a requirement to deploy 12 separate microservices.

M01 | R01-R03

Access and consent

modules/access/

M02 | R04-R06

Health profiles

modules/profiles/

M03 | R07-R10

Case lifecycle

modules/cases/

M04 | R11-R14

Adaptive intake

modules/intake/

M05 | R15-R17

Urgency and scope

modules/safety/

M06 | R18-R20

Document processing

modules/records/

M07 | R21-R23

Evidence retrieval

modules/evidence/

M08 | R24-R27

Guidance generation

modules/guidance/

M09 | R28-R31

Doctor matching

modules/providers/

M10 | R32-R33

Appointment summary

modules/exports/

M11 | R34-R36

Privacy lifecycle

modules/privacy/

M12 | R37-R40

Operations + evaluation

modules/operations/

One owner per behavior

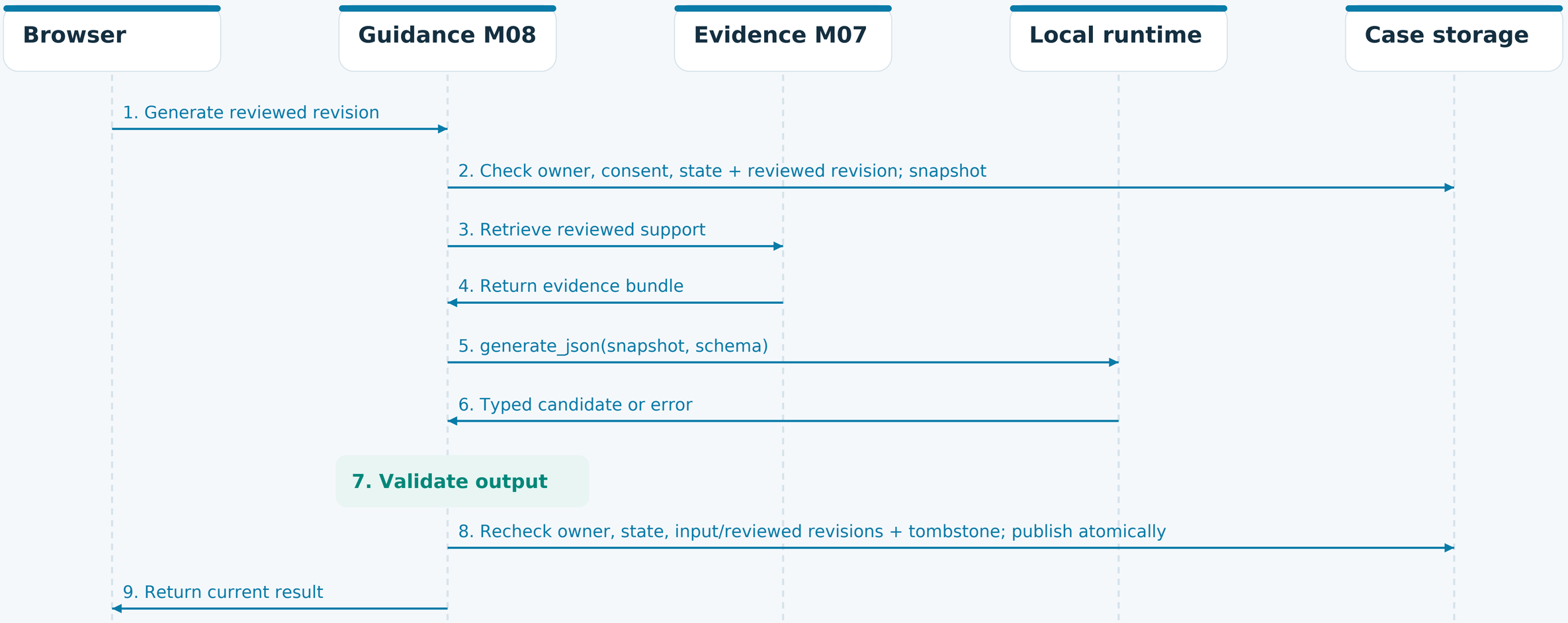
Screens render interaction. Domain services own decisions. Shared helpers inherit their owning requirement IDs.

Source: PMD v2.0 p. 18 | Planned package paths under apps/api/app/modules/

AWARE CARE

Generation is a guarded transaction

Immutable reviewed snapshot; no model tools or internet. Initial capacity: 1 active generation job + queue of 3.



Fail without publishing unsafe or stale output

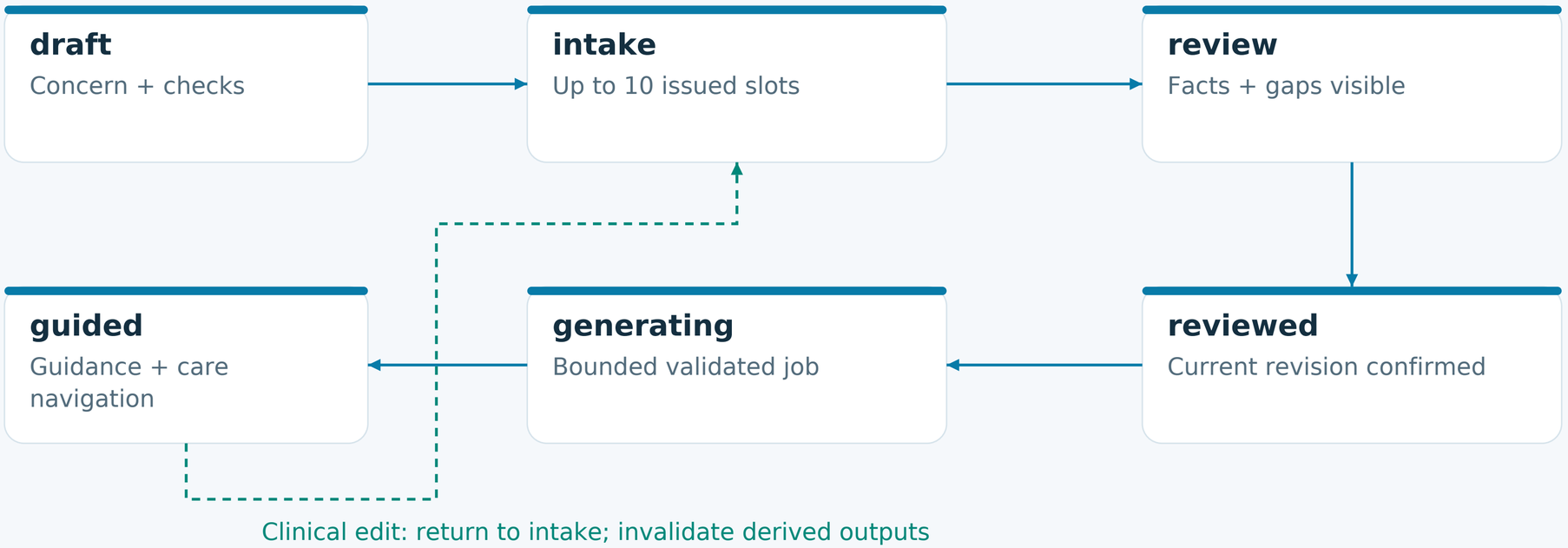
1 schema repair allowed; total generation deadline 60 seconds. Failed validation gives a limited fallback. Edits invalidate pending output.

Source: PMD v2.0 pp. 23, 29-30, 34 | All operations shown are proposed

AWARE CARE

Case state is the authority

Primary success path plus overriding guards. This plate summarizes transitions; the PMD state table remains normative.



OVERRIDING GUARD

Urgent concern

Enter urgent. Block routine intake, normal guidance and doctor search. Restart requires a new case.

LIMITED

Scope or service limits

Unknown critical facts, unsupported scope or required-service failure constrain the result. Retry only when recoverable.

DELETION

deleting → deleted

Block access immediately; cancel jobs and purge stores. Late workers check the tombstone. No return to active.

QUESTION ACCOUNTING

The cap is on issued slots

Retry or edit an existing answer adds no slot. An invalidated issued prompt still counts. No 11th question.

Source: PMD v2.0 pp. 25-27, 30, 32 | Clinical edits never reuse an old result as current

AWARE CARE

Separate patient facts from clinical evidence

Both stay local during core processing. Confirmation establishes case facts; reviewed provenance supports clinical claims.

PATIENT RECORD LANE | M06

Validate file
Type, bytes, pages and limits

→

Extract locally
Bounded worker; no network

→

User confirms facts
Confirm, correct or reject

→

Confirmed context
Fact IDs + current revision

CLINICAL EVIDENCE LANE | M07

Approved sources
Presenter-controlled ingest

→

Review + version
Publisher, date, hash, expiry

→

Retrieve locally
Keyword + embedding search

→

Validate support
Source ID AND claim support

RETRIEVAL BUDGET

Bounded evidence

Up to 8 candidates; retain up to 5 relevant passages within 6,000 evidence tokens. Values require model-context testing.

OUTPUT RULE

No unsupported claims

Clinical claims cite approved evidence. Patient statements reference confirmed facts. Missing support produces a limited result.

Source: PMD v2.0 pp. 19-20, 28-30 | Reviewed sources alone do not prove claim support

AWARE CARE

Eligibility first. Explained ranking second.

M09 uses structured directory facts. The LLM neither invents providers nor determines their ranking.

1. Current guidance

Non-urgent care setting and reviewed specialty recommendation

2. Hard constraints

Specialty, visit type, radius or telehealth jurisdiction; required plan/language

3. Deterministic rank

Relevance, optional preferences, freshness, distance, stable ID

4. Explain matches

Confirmed matches, unknowns, source dates and contact link

REQUIRED CRITERIA

Unknown does not qualify

An unverified required insurance plan or language excludes the record. Insurance acceptance is not proof of participation in the exact plan.

ZERO RESULTS

Ask before relaxing a filter

Explain the exclusion. Propose 1 explicit change. Rerun only after the user accepts. Never silently broaden eligibility.

Contact is a deliberate handoff

Use approved HTTPS links without consultation payload. Availability, credential status and exact plan participation need separate verification.

Source: PMD v2.0 pp. 21, 31 | Requirements R28-R31 | No automatic booking

AWARE CARE

Privacy is a lifecycle, not a location label

Local processing must be accompanied by authorization, retention, controlled egress and verifiable deletion.

1. Authorize
Consent + owner + role

→

2. Store locally
Restricted DB and files

→

3. Use by revision
No stale guidance or exports

→

4. Expire or delete
Revoke, cancel and purge

GUEST CONTENT

30 minutes idle / 8 hours max

Expiry at whichever comes first. Reset purges health content. Planned browser memory is temporary, with no health data in localStorage.

SAVED + EXPORTED DATA

30 days / 15 minutes

Saved synthetic cases default to 30 days, with 7-day option. Server export files expire after 15 minutes. Cleanup runs every 5 minutes.

DELETION COMPLETENESS

No late-job resurrection

Block access, revoke downloads, cancel jobs and purge dependent stores. Report completion only after removal acknowledgements.

Boundary that must remain visible

A downloaded browser file is outside server deletion. No automatic clinician messaging, EHR write, or cloud-model fallback is included.

Source: PMD v2.0 pp. 29, 32 | Defaults belong to the planned local demo, not the browser mock

AWARE CARE

Trace behavior from specification to evidence

Mappings are bidirectional and may be many-to-many. Shared code does not require duplicate product rules.

Requirement R30
Explained, repeatable provider ordering

→

Screens A10 + A11
Doctor matches and details

→

Owner M09
ProviderSearchResult
ProviderMatch

→

Operations
searchProviders
getProvider

PROPOSED IMPLEMENTATION

Owning service + screen files

apps/api/app/modules/providers/service.py
apps/web/src/screens/A10.tsx
apps/web/src/screens/A11.tsx

PROPOSED VERIFICATION

Acceptance + UI tests

tests/acceptance/test_R30.py
tests/ui/A10.spec.ts; tests/ui/A11.spec.ts
Evidence identifies the tested commit and build.

The release packet connects the proof

Spec and contract hashes + application commit + device/model/corpus manifests + executed tests + network and deletion evidence.

CI checks IDs, paths, coverage and contract drift. Meaningful assertions and human review establish behavioral correctness.

Source: PMD v2.0 pp. 18, 31, 42-43 | Paths and tests are planned, not verified implementations

AWARE CARE

Demonstrate the workflow. Verify the deployment.

Separate mock-data proof, local runtime proof and real-world outcome evidence.

NOW

Mock-data experience

Synthetic patient journeys, scripted guidance, fictional providers and explicit failure states.

Do not infer live AI, privacy controls or savings.

NEXT

Connected local demo

Actual ZGX inventory, pinned runtime, reviewed corpus, internal stores and controlled retrieval.

Measure performance and verify access, egress and reset.

LATER

Healthcare edge pilot

Deploy close to the care workflow with approved intended use, evaluation and operating ownership.

Validate clinical outcomes, workload and total cost.

5-MINUTE EXECUTIVE STORY

0-1 min	1-2 min	2-3 min	3-4 min	4-5 min
Concern + data choices	Record fact + intake	Guidance + evidence	Doctors + summary	Runtime + reset

Target gate: 10 full device rehearsals, including recovery. Targets of 5-second questions and 20-second guidance are acceptance goals, not established performance.

Source: PMD v2.0 pp. 4-5, 34-36, 44 | No outcome or hardware performance claims implied